

Parkinson's Disease Care Summary

Robert Wells | Prepared by Stephanie Wells | December 11, 2026

Patient	Robert Wells
Date of birth	March 15, 1968
Address	Fremont, CA
Primary caretaker	Linda Wells, (510) 555-0193
Secondary contact	Stephanie Wells, stephanie.wells@Finthesiss.ai, (415) 555-0187
Neurologist	Dr. Steven Mitchell, Fremont Neurology Associates
Neurologist phone	(510) 555-0376
Diagnosis	Parkinson's disease, Stage 2
Diagnosis date	February 2024
This summary	CARE-RW-2026-12, prepared for Dr. Mitchell quarterly visit

1. Summary of Condition Since Last Visit (October 16, 2026)

Robert Wells continues in Stage 2 Parkinson's with gradual but manageable progression. Since the October 16 visit, tremor frequency has been stable on most days, with occasional higher-tremor mornings noted in the symptom log (X04.xlsx). Balance has shown a slight downward trend, with one near-fall in the hallway recorded on February 16, 2026 and no falls since. Linda Wells reports that the morning off period before the first Sinemet dose takes effect (approximately 45 minutes post-dose) remains the highest-risk window for falls. The family installed grab bars in the bathroom in November 2026 and the stair rail in October 2026.

Robert attends PT twice per week (provider confirmed by Dr. Mitchell in October). Mood is generally positive; he reports enjoying the morning crossword and daily short walks of 10 to 15 minutes when weather permits. Sleep quality remains variable; Linda reports increased nocturnal restlessness approximately three nights per week. No significant swallowing difficulty noted; appetite stable.

2. Medication Log

Medication	Dose	Frequency	Notes
Sinemet (carbidopa-levodopa)	25/100mg	Three times daily	8 AM, 1 PM, 7 PM; adjusted Feb 2026
Melatonin	3 mg	Bedtime (PRN)	For nocturnal restlessness
Vitamin D3	2000 IU	Daily	Bone health, falls prevention
Multivitamin	1 daily	Morning	With breakfast

The Sinemet timing was adjusted at the February 20, 2026 visit and the response was reaffirmed at the October 16, 2026 visit. Effect onset is approximately 35 to 45 minutes post-dose. Off periods are most pronounced in the morning before the first dose. Stephanie and Linda track medication timing in the family symptom log (X04.xlsx) for Dr. Mitchell.

3. Symptom Trend Summary (January 2026 to November 2026)

Period	Avg Tremor (0-5)	Avg Mobility (0-5)	Avg Balance (0-5)	Falls	Notes
Jan 2026	2.5	3.0	2.8	0	Stable
Feb 2026	2.9	2.9	2.5	1 near-fall	Dose adjusted
Oct 2026	2.4	3.1	2.9	0	Post-dose adjustment, improved
Nov 2026	2.6	3.0	2.8	0	Grab bars installed

Detailed daily symptom entries for January and February 2026 are recorded in X04.xlsx; the October and November figures above are visit-summary averages compiled from updates provided by Linda. Stephanie

maintains this record remotely using data provided by Linda.

4. Home Modification Status

Modification	Status	Installed Date	Notes
Bathroom grab bars (x3)	Complete	November 2026	Toilet and shower
Stair rail (first floor)	Complete	October 2026	Right side descending
Shower chair	Complete	January 2026	Teak, purchased online
Night light (hallway)	Complete	November 2026	Motion-activated
Non-slip bath mat	Complete	January 2026	Suction-cup base
Bed rail	Pending	Q1 2027 target	For nocturnal restlessness
Ramp for front step	Evaluating	TBD	2-inch threshold, may not need

5. Physical Therapy Progress

Robert attends PT twice per week at Fremont Physical Therapy Center. The current program focuses on gait training, balance exercises, and upper-body strength to reduce falls risk. Linda attends one session per month to learn assisted-exercise techniques for home use. Stephanie's cousin Amy Davis (medical resident at Crestwood Memorial) reviewed the PT plan in January 2026 and confirmed it is appropriate for Stage 2. The PT provider has been asked to share session notes with Dr. Mitchell.

6. Questions and Agenda for December 11, 2026 Visit

1. Review the Sinemet dose response: is the current 25/100mg three-times-daily schedule still appropriate given the improved mobility trend? 2. Nocturnal restlessness: consider whether a low-dose Sinemet CR extended-release dose at bedtime would improve sleep quality. 3. Falls prevention: confirm whether a bed rail is advisable given the restlessness pattern. 4. Next-stage planning: at what symptom trajectory does Stage 3 typically occur, and what modifications should the family begin preparing? 5. Caretaker support: what respite care and support-group resources does Fremont Neurology Associates recommend for Linda? 6. PT referral renewal: confirm PT twice per week continues to be prescribed. 7. Next quarterly visit: request a March 2027 slot.

7. Family Contacts for Emergency Coordination

Role	Contact
Primary caretaker	Linda Wells, (510) 555-0193, linda.wells@gmail.com
Daughter (secondary)	Stephanie Wells, (415) 555-0187, stephanie.wells@Finthesiss.ai
Son	Kyle Wells, (206) 555-0216, kyle.wells@gmail.com
Neurologist	Dr. Steven Mitchell, (510) 555-0376, Fremont Neurology Associates
Medical resident (cousin)	Dr. Amy Davis, (408) 555-0245, Crestwood Memorial Bay Area
Emergency	911, then Linda, then Stephanie

This document is a private family summary prepared for Dr. Mitchell's quarterly review. It does not constitute medical advice. See X04.xlsx for the symptom log and P05.pdf (Stephanie's annual physical) for family health context.